

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: Observational Study Protocol: LIVER-R (A global real-world study of durvalumab-based regimens in patients with hepatobiliary cancers) **Short Title/Acronym:** LIVER-R **Protocol Number:** D419CR00035 **NCT Number:** NCT06252753 **Sponsor Name:** AstraZeneca **Investigational Product:** Durvalumab-based combination therapies **Phase of Development:** N/A (Observational Study) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effectiveness of durvalumab-based regimens in real-world settings as measured by real-world overall survival (rwOS). **Secondary Objectives:** To evaluate real-world clinical endpoints including duration of treatment, progression-free survival (rwPFS), time to treatment progression, time to next treatment, recurrence-free survival, treatment patterns, and the incidence of clinically significant safety events of interest.

2.2 Background & Rationale Durvalumab-based regimens have shown improved outcomes compared with the standard of care in unresectable hepatocellular carcinoma (uHCC) and advanced biliary tract cancer (aBTC) in recent phase 3 clinical trials (e.g., HIMALAYA, TOPAZ-1). Given the anticipated launch and use of these treatments globally, there is a distinct medical necessity to capture contemporary real-world data. This study will produce a large, global, standardized dataset representing day-to-day clinical practice to better understand the long-term outcomes and safety profiles of durvalumab-based therapies outside of strictly controlled clinical trial environments.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Ambispective, multi-country, multi-center) **Control Type:** Single-arm (Descriptive, noncomparative) **Blinding:** Open-label (Routine clinical practice) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 2,500 (1,135 with uHCC and 1,355 with aBTC), with potential expansion up to 4,490 patients. **Number of Sites:** ~179 sites globally (across 22 countries in North and South America, Europe, the Middle East, and the Asia-Pacific region). **Estimated Study Start:** 16-Mar-2023 **Estimated Primary Completion:** 30-Dec-2030

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years and considered a lawful adult in the respective country at the index date. 2. Confirmed presence of malignancy of primary hepatobiliary cancer (i.e., uHCC or aBTC) by the treating physician. 3. Type of hepatobiliary cancer indication is approved to be treated with a durvalumab-based regimen in the respective country, or the regimen is administered as part of an Early Access Program (EAP). 4. Informed consent obtained as per country-level regulations.

4.2 Exclusion Criteria 1. Currently participating, previously participated, or planning to participate in any clinical trial for investigational treatment of hepatobiliary cancers on or after the diagnosis date until the index date. 2. Received other systemic therapies for the hepatobiliary cancer indication prior to initiating the durvalumab-based regimen. 3. Received a liver transplant during the baseline period.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Durvalumab-based combination therapies administered at the physician's discretion in accordance with routine clinical practice or EAP. **Route of Administration:** Intravenous (Standard for durvalumab regimens) **Duration of Treatment:** Variable, up to a maximum follow-up of 2 years for patients with aBTCs or 3 years for patients with uHCC.

5.2 Concomitant & Prohibited Therapy Permitted: Routine supportive and concomitant medications per standard local clinical practice guidelines. **Prohibited:** Other concurrent investigational systemic therapies for hepatobiliary cancers.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Baseline	Up to 5 years pre-Index	Retrospective extraction of demographic, clinical, and medical history data from medical records.
Index Date	Day 0	Initiation of the first-line durvalumab-based regimen.
Routine Follow-Up	Every 6 months	Data collection at 6-month intervals including rWOS, rWPFS, treatment patterns, and clinically significant events.
End of Study	Up to Year 2 (aBTC) or Year 3 (uHCC)	Final data extraction on survival status, cause of death, or study withdrawal/loss to follow-up.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Real-world Overall Survival (rwOS). Calculated as the time from the index date to the date of death by any cause. Assessed at 6, 12, 24, and 36 months.

Measurement Method: Secondary data collection from electronic medical records and primary data collection during routine visits.

7.2 Secondary & Exploratory Endpoints Real-world Progression-Free Survival (rwPFS) at 6, 12, 24, and 36 months. Time to treatment progression (rwTTP) and Time to next treatment (rwTTNT). Treatment duration, treatment patterns, and safety/adverse event assessments.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collection of negative side effects and clinically significant safety events of interest recorded during the 6-month interval reviews of patient medical charts. **Serious Adverse Events (SAEs):** Reported per standard local observational study and pharmacovigilance regulations. **Data Monitoring Committee (DMC):** AstraZeneca internal safety monitoring tailored for real-world/observational datasets.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All enrolled patients with valid baseline and follow-up data. Descriptive analyses will be conducted separately for each primary hepatobiliary cancer indication (uHCC and aBTC). **Sample Size Justification:** The target enrollment of ~2,500 to 4,490 patients provides a robust, standardized real-world global dataset. As this is a descriptive, noncomparative study, no formal hypotheses will be tested. **Handling of Missing Data:** Time-to-event outcomes (e.g., rwOS, rwPFS) will be analyzed using the Kaplan-Meier method, appropriately censoring patients lost to follow-up or withdrawn.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local ethical regulations for observational, non-interventional research.

Monitoring Plan: Intermittent verification of secondary data extracted from patient medical charts against source data where primary collection occurs. **Audit Trail:** Secure, standardized global real-world database electronic case report forms (eCRF) implemented across all 179 global sites.

11. Study Identifiers & Governance

Primary ID: D419CR00035 **Registry Number:** NCT06252753 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** LIVER-R **Official Regulatory Title:** Observational Study Protocol: LIVER-R

12. Clinical Framework (Hepatobiliary Cancer Specific)

Primary Condition: Unresectable Hepatocellular Carcinoma (uHCC) & Advanced Biliary Tract Cancers (aBTC) **Diagnosis Threshold:** Confirmed malignancy of primary hepatobiliary cancer by treating physician. **Medical Methodology:** Routine clinical practice / Real-world evidence (RWE) guidelines. **Optimization Target:** Improved real-world Overall Survival (rwOS) utilizing durvalumab-based therapy.

13. Study Procedures & Methodology

Management Pathway: Standard of care/routine clinical practice for uHCC and aBTC. **Education Protocol (HCP):** Standard observational study site initiation and registry tracking training. **Education Protocol (Patient):** N/A (Standard of care counseling provided by the treating physician). **Quality Audit Mechanism:** Periodic cross-referencing of standardized dataset entries with patient medical charts.

14. Observation & Follow-Up Schedule

Total Duration: Maximum 2 years (aBTC) or 3 years (uHCC). **Onsite Visit Cadence:** Unstructured; dictates follow real-world routine clinical visit schedules. **Remote Monitoring Frequency:** N/A (Chart reviews conducted). **Checklist Review Interval:** Data collected at baseline and at predefined 6-month intervals.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				